ICAB March – Jun-July 2025 Exam
Case Study
Model Answer:

Draft Report

To : Board of Director, Astha General Hospital Limited (Astha)

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Date : 27 August 2025

Subject : Report on Assessment of business operation and performance, advising on accounting & tax issues, regulatory matters, evaluation of strategic options, suggestion of investing and financing options, and analysis of ethical issues and sustainability factors of the business.

Terms of Reference:

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1. Executive Summary:

1.1. Adjustment schedule with explanations on reporting & compliance issues

Astha has made significant financial adjustments to improve compliance and transparency. Gratuity obligations, previously unrecognized under the Labor Act, 2006, have now been incorporated through restated financial statements, reflecting annual costs, long-term liabilities, and related tax impacts. Land revaluation gains were reclassified from non-operating income to "Other Comprehensive Income," reducing profits but strengthening equity presentation. Workers' Profit Participation Fund (WPPF) was recalculated based on corrected profits, ensuring fair employee allocations. Regulatory fines, professional fees, property damage, and lawsuit provisions have been recognized, while contingent liabilities for malpractice remain disclosed.

These corrections lowered profit before tax to approximately BDT 171 million, though no further current tax provision is required beyond the minimum tax already recognized. Deferred tax impacts are material, with liabilities from revaluation offset partly by assets from gratuity and provisions. Overall, while profitability declined, equity reporting is more robust, enhancing compliance, accuracy, and stakeholder confidence.

Recommendation to the board:

- Implement a structured gratuity obligation recognition process with annual actuarial valuations to ensure employee benefit obligations are consistently recognized and tax implications accurately reflected.
- Conduct periodic revaluations of land and assets, properly routing gains through "Other Comprehensive Income" and equity reserves to maintain compliance and transparency.
- Establish proactive checks for regulatory requirements, such as staff credential verification, to minimize fines, penalties, and reputational risks.
- Maintain a litigation and claims register to distinguish between provisions and contingencies, ensuring accurate recognition of potential liabilities.
- Introduce insurance coverage and stronger physical security to protect property and reduce financial exposure to vandalism or unforeseen incidents.
- Align tax reporting with accounting adjustments and reinforce internal audit reviews to detect misclassifications, omissions, and errors early.

1.2. Evaluation of financial and non-financial performances of Astha General Hospital Limited.

In 2025, Astha General Hospital Limited achieved a strong financial recovery, reflecting resilience amid challenging economic and political conditions. Revenue increased by 13.2% to BDT 3,980.67 million, driven by higher patient inflow, improved diagnostic services, and consistent pharmacy performance. Although slightly below the healthcare industry's 15–20% average growth, the result marks steady expansion in core medical operations. Cost of revenue rose by 11.1%, showing effective cost control despite inflationary pressures. Consequently, gross profit grew by 14.7% to BDT 2,365.86 million, while the gross profit margin improved slightly to 59.43%, indicating efficient pricing and operational management.

After adjustments, net profit surged to BDT 123.49 million from BDT 3.61 million, raising the net profit margin to 3.10%. Liquidity indicators softened slightly, with the current ratio declining to 1.39 and the quick ratio to 1.14, suggesting moderate working capital tightening. The debt-to-equity ratio fell to 0.02, highlighting minimal reliance on borrowings, while the Times Interest Earned ratio improved to 3.93, demonstrating a stronger debt-servicing capacity. Return on Assets (0.97%) and Return on Equity (1.12%) also strengthened, reflecting profitability recovery and improved operational performance. Non-financially, Astha advanced in digital transformation, service quality, and community health programs but continues to face challenges in workforce retention, sustainability investment, and governance oversight. Overall, Astha's strengthened financial position, disciplined cost management, and expanding healthcare network provide a strong foundation for sustained growth and long-term institutional stability.

Recommendation to the board:

- Diversify service offerings by expanding specialized care units, introducing advanced diagnostic and preventive health packages, and scaling telemedicine services to enhance revenue and market reach.
- Improve cash flow and liquidity management through faster receivable collection, tighter working capital control, and prudent use of short-term financing to support sustainable growth.
- Enhance asset utilization and return on investment by increasing patient throughput, optimizing bed occupancy, and ensuring full operational use of new facilities and equipment.
- Standardize employee training and retention programs by introducing structured career development pathways, performance-based incentives, and continuous professional education to reduce turnover and improve service consistency.
- Strengthen corporate governance and internal controls by ensuring auditor independence, expanding internal audit coverage, and enhancing board oversight to maintain transparency, compliance, and investor confidence.

1.3. SWOT Analysis

Astha General Hospital Limited has emerged as one of Bangladesh's most trusted private healthcare institutions, recognized for its commitment to quality medical care, diagnostic excellence, and patient-centric services. With a strong brand presence, an expanding service portfolio, and integration of technology through digital health records and telemedicine, Astha continues to strengthen its position in the competitive healthcare sector. The organization's steady revenue growth, strong equity base, and disciplined cost management reflect financial resilience amid economic volatility and sectoral challenges.

Despite these strengths, Astha faces notable internal challenges, including weak internal controls, delays in financial adjustments, and inconsistent service quality across newly expanded units. Workforce turnover, ethical concerns in HR practices, and limited sustainability initiatives further constrain operational excellence. Additionally, the slow pace of digital transformation and underutilization of asset capacity limit efficiency and patient outreach potential.

Astha has substantial opportunities to accelerate growth by expanding regionally through franchise-based or satellite healthcare models, scaling up telemedicine and automation, and investing in renewable energy and sustainable waste management. Leveraging these opportunities would enhance efficiency, reduce costs, and align Astha with emerging ESG and healthcare technology standards.

Externally, the hospital must navigate persistent threats such as inflation, rising operational costs, supply chain vulnerabilities, and increasing competition from both new private hospitals and informal healthcare providers. Strengthening governance, enhancing technology adoption, and investing in workforce development will be essential for maintaining service quality, sustaining profitability, and achieving long-term institutional stability in Bangladesh's evolving healthcare landscape.

Recommendation to the board includes:

- Strengthen internal controls and reporting systems to ensure transparency, compliance, and investor confidence in line with IFRS and regulatory standards.
- Expand the digital healthcare strategy through telemedicine, integrated patient apps, and enhanced online services to boost accessibility and engagement.
- Invest in automation and digitization across operations to reduce costs, minimize errors, and improve overall efficiency.
- Pursue a franchising and partnership-based expansion model to enter tier-2 cities and underserved areas with limited capital investment.
- Strengthen workforce capability and governance through regular training, standardized procedures, and fair practices to uphold quality and reputation.

1.4. NPV analysis, suggestion to board and comment on compatibility with mission & vision:

The investment evaluation presents two viable opportunities for Astha General Hospital Limited: the acquisition of Fair Hospital Limited and the establishment of Astha Healthcare Network Limited. Both options generate positive Net Present Values (NPVs), indicating financial feasibility. However, the acquisition of Fair Hospital Limited delivers a higher NPV of BDT 134.10 million, compared to BDT 89.65 million from the healthcare network initiative. The acquisition offers immediate strategic and operational advantages, including faster market entry into Chattogram, access to an established patient base, and potential cost efficiencies through shared resources and infrastructure. This investment directly supports Astha's mission to deliver ethical, quality, and affordable healthcare services nationwide.

The proposed Astha Healthcare Network presents an innovative, collaborative model that aligns with Astha's long-term vision of building a digitally connected and inclusive healthcare ecosystem. Its asset-light structure and partnership-based approach have significant future potential; however, the model requires strong operational capacity and governance mechanisms to ensure consistent service quality across member facilities. Considering the comparative financial outcomes, risk profile, and strategic alignment, it is recommended that the Board proceed with the acquisition of Fair Hospital Limited as the immediate growth priority while retaining the healthcare network initiative as a structured long-term diversification strategy.

Recommendation to the board includes:

- Proceed with the acquisition of Fair Hospital Limited, as it presents the highest NPV (BDT 134,104,971) and delivers superior financial and strategic returns.
- Defer the establishment of Astha Healthcare Network Limited for future implementation, once stronger operational and financial capacity is achieved.
- Integrate Fair Hospital Limited swiftly and effectively, ensuring alignment with Astha's systems, culture, governance, and service quality standards.
- Leverage Fair Hospital's infrastructure and market position to accelerate expansion into Chattogram and enhance nationwide brand presence.
- Allocate resources to upgrade Fair Hospital's facilities and technology, ensuring consistency with Astha's commitment to ethical, high-quality, and patient-centric healthcare.
- Implement a post-acquisition monitoring framework with clear KPIs to evaluate financial performance, operational integration, and service outcomes.

1.5. Evaluation of viability of bank loan as financing option:

The evaluation of bank financing for the acquisition of Fair Hospital Limited confirms that full debt funding is both financially viable and strategically beneficial for Astha. The project delivers a positive NPV of BDT 103.7 million, even after accounting for all financing costs, indicating strong value creation. Astha's low gearing provides capacity for sustainable borrowing, while the proposed 13% syndicated loan ensures access to the required capital. The structured repayment of one-fifth of the principal annually supports predictable debt servicing and prudent cash flow management.

This financing structure enhances capital efficiency and shareholder returns by leveraging debt while preserving equity for future initiatives. To ensure stability, Astha should maintain a clear debt servicing plan, build a liquidity reserve for repayments, and regularly monitor cash flows. With disciplined financial oversight and careful covenant management, the company can achieve its growth objectives while maintaining a strong balance sheet and financial resilience.

Recommendation to the board:

- Proceed with bank financing for the acquisition of Fair Hospital Limited, as the debt-funded structure generates a positive NPV of BDT 103.7 million and enhances shareholder value.
- Leverage Astha's low gearing position to utilize full debt financing, optimizing capital efficiency while preserving internal equity for future strategic investments.
- Implement a comprehensive debt servicing plan to ensure timely payment of interest and smooth annual principal repayments over the five-year loan term.
- Establish a liquidity reserve or sinking fund from project cash flows to support repayment obligations and safeguard financial stability.
- Monitor Fair Hospital's cash flows on a quarterly basis to confirm debt coverage capacity and detect potential liquidity or financing risks early.
- Negotiate favourable loan covenants and repayment terms to maintain operational flexibility and avoid restrictive financial conditions during the loan period.

1.6. Expected share premium in IPO and number of new shares to be issued:

Astha General Hospital Limited plans to raise BDT 4,000 million through an Initial Public Offering (IPO) by issuing up to 25% of its shares, subject to BSEC approval. The board initially proposed a 500% share premium (BDT 50 over face value), resulting in an issue price of BDT 60 per share. However, based on BSEC's fair valuation method, which equally weighs NAV, EPS, and sectoral P/E, the proposed premium appears overstated. With an NAV of BDT 55.78, EPS of BDT 4.07, and a sectoral P/E ratio of 12.2, the fair value per share is estimated at BDT 58, suggesting a reasonable premium of BDT 48 (480%).

At this revised valuation, Astha would need to issue around 98.52 million shares to raise the target amount, increasing the total share base to 327.87 million. This would give public investors 30.05% ownership, exceeding the board-approved 25% limit. To ensure regulatory compliance and investor confidence, Astha should revise the premium and adjust the issue size accordingly. With these adjustments, the IPO remains a financially viable and strategically sound option for funding expansion while maintaining governance standards and shareholder value.

Recommendation to the board:

- Revise the proposed share premium to around BDT 48 per share (480%), ensuring alignment with fair valuation metrics and improving the likelihood of BSEC approval.
- Conduct an independent fair value assessment through a credible valuation firm to validate the proposed pricing, enhance transparency, and strengthen the justification for the share premium.
- Engage a BSEC-accredited issue manager and underwriter early in the process to ensure proper documentation, effective regulatory coordination, and credible valuation presentation.
- Develop a detailed utilization plan for IPO proceeds, outlining project-specific fund allocation, timelines, and expected returns to demonstrate accountability and strengthen investor trust.
- Enhance financial reporting and internal control systems ahead of the IPO to improve transparency, reduce audit risks, and comply with BSEC's public entity standards.
- Improve corporate governance by appointing independent directors and establishing an audit committee, reinforcing oversight and meeting public listing governance requirements.

1.7. Ethical issues involving Astha's existing and planned business:

Astha General Hospital Limited faces a range of ethical, governance, and compliance challenges that require strategic corrective action to preserve its reputation and ensure long-term sustainability. While the hospital has demonstrated steady operational growth and strong brand trust, several practices fall short of acceptable ethical and regulatory standards.

The hospital's appointment of a non-FRC enlisted auditor, with potential links to internal management, raises questions about audit independence and adherence to statutory compliance expected from a Public Interest Entity. In parallel, the absence of clear data protection protocols for patient information within the Hospital Management System and "Astha Card" program presents significant privacy and confidentiality risks.

Operationally, disparities in staff training and overreliance on overworked employees compromise service quality and workplace ethics. Governance risks persist due to concentrated decision-making among family members without independent oversight, undermining transparency and accountability. Weak supplier due diligence and unequal risk-sharing arrangements in procurement practices expose smaller vendors to financial and ethical vulnerabilities.

Additionally, limited coverage of health and safety audits, coupled with aggressive expansion despite liquidity pressures, reflects potential neglect of employee well-being and financial prudence. Addressing these issues through stronger governance, ethical sourcing, transparent reporting, and comprehensive data protection measures will be critical for maintaining stakeholder trust and sustaining Astha's growth trajectory in Bangladesh's healthcare sector.

Recommendation to the board

- Appoint an FRC-enlisted external auditor to ensure full compliance with regulatory requirements and maintain transparency. Disclose auditor credentials and independence status to reinforce the credibility of Astha's financial statements.
- Introduce independent board members and establish an audit and ethics committee to mitigate governance risks arising from family dominance, ensuring accountability, balanced oversight, and adherence to good corporate governance practices.
- Develop and implement a comprehensive patient data protection and privacy policy governing the Hospital Management System and Astha Card program, ensuring informed consent, secure data handling, and compliance with confidentiality standards.
- Standardize clinical and administrative staff training programs across all branches and establish performance benchmarks to ensure consistent service quality, ethical patient care, and equitable human resource development.
- Establish a supplier code of conduct covering ethical sourcing, fair pricing, labor standards, and sustainability practices. Revise consignment terms to create equitable risk-sharing arrangements with SME vendors and long-term partners.
- Align expansion and investment strategy with financial capacity by slowing aggressive growth plans, evaluating the performance of existing facilities, and maintaining financial prudence to safeguard stakeholder interests and long-term sustainability.

1.8. Assessment of sustainability factors of Astha's business:

Astha General Hospital Limited demonstrates steady progress in economic and social performance but faces notable challenges in human and environmental sustainability. The hospital's strong revenue growth, prudent financial management, and low debt levels reflect a stable economic foundation. However, modest profitability, underutilized assets, and tightening liquidity highlight the need for improved financial efficiency and strategic investment in technology and capacity enhancement.

From a human sustainability perspective, high staff turnover, inconsistent training, and the absence of structured well-being and mental health initiatives undermine workforce stability. Limited diversity and inclusion policies further restrict opportunities for women and differently abled professionals, weakening Astha's potential to build a resilient, equitable workplace.

Socially, Astha contributes meaningfully through community health programs, vaccination drives, and affordable care, yet these initiatives lack measurable outcomes and transparent reporting. Its urban-focused expansion also limits rural healthcare access, constraining its broader social impact.

Environmental sustainability remains the weakest area. Although basic waste management practices are in place, high energy consumption, absence of renewable energy adoption, and inadequate wastewater systems demonstrate limited environmental responsibility.

Recommendation to the board

- Strengthen human sustainability through structured workforce development programs, focusing on continuous training, fair workload management, and clear career progression to reduce turnover and enhance service quality.
- Enhance workplace health, safety, and well-being systems by conducting regular safety audits, investing in modern protective infrastructure, and introducing mental health and stress management programs for healthcare staff.
- Promote social inclusivity by expanding healthcare access to rural and semi-urban areas, leveraging telemedicine, mobile health units, and partnerships with local providers to ensure equitable healthcare delivery.
- Adopt an environmental sustainability framework incorporating renewable energy use, improved biomedical waste management, and water conservation measures to minimize the hospital's ecological footprint.
- Establish a dedicated ESG and Sustainability Committee to oversee environmental, social, and governance initiatives, ensure transparent sustainability reporting, and align operations with national and international healthcare sustainability standards.

2. Adjustment schedule with explanations on reporting & compliance issues. (Requirement-a-i)

2.1. Schedule of adjustments required for current reporting and compliance issues.

Gratuity Liability:

Astha have not recognized gratuity obligation in the past. However, it now has performed actuarial valuation of the gratuity obligation for recognition. Because gratuity obligation has not been recognized in the past, Astha now will have to restate its financial position.

From the actuarial report, Service costs should be reflected in the administrative expenses and interest expenses should be reported in finance expense of their respective year. Remeasurement loss should be reported in Other comprehensive income and within Remeasurement reserve under equity. Finally Closing plan obligation should be reported in non-current liabilities of each year. Opening plan obligations of 2024 should be reported in the comparative of opening balance.

To restate and adjust the financial statements, following entry will be required:

To recognize liabilities at the beginning of comparative year (01 July 2023):

Retained Earnings	Dr.	13,889,462	
Gratuity Liabilities	Cr.		13,889,462

To recognize expense & liabilities for the year ended in 2024:

Retained Earnings (Service Cost)	Dr.	2,222,314	
Retained Earnings (Interest Exp)	Dr.	833,368	
Re-measurement reserve	Dr.		
Contract Liabilities	Cr.		3,310,334

To recognize liabilities and expenses related to current year (2025):

Service Cost	Dr.	2,788,992	
Interest Expense	Dr.	1,289,985	
Re-measurement reserve	Dr.	2,467,807	
Contract Liabilities	Cr.		6,546,784

Land revaluation gain:

Revaluation gain on land is an OCI item and should be presented as Revaluation Surplus under Equity and reported in Statement of Other Comprehensive Income instead of Statement of Profit or Loss. This means, revaluation gain will be removed from non-operating income and reported in Other Comprehensive Income.

As the WPPF has been calculated based on profit including the revaluation gain, it should be reversed and new WPPF expense should be calculated on revised profit after considering all necessary adjustments. Therefore, following entry will be needed:

Non-operating income	Dr.	853,625,465	
Revaluation Surplus	Cr.		853,625,465
Provision for WPPF	Dr.	49,546,630	
WPPF Expense	Cr.		49,546,630

Revised WPPF expense will be calculated before later part of this adjustment schedule.

Penalty by mobile court:

Astha was penalized by the mobile court for BDT 500,000 due to its failure to verify staff credentials. Subsequently, Astha appointed a forensic auditor with a fee of BDT 350,000 to validate all doctors' credentials. These transactions have not been recognized in the draft financial statements. To recognize these transactions, following entry will be required:

Fines & Penalty Expenses	Dr.	500,000	
Provisions & Accruals	Cr.		500,000
Professional Fees	Dr.	350,000	
Provisions & Accruals	Cr.		350,000

Astha expects potential lawsuits up to BDT 1,500,000 but estimates probability to be less likely than not (i.e. less than 50%). Therefore, no provision is required for these expected lawsuits. However, these should be reported as contingent liabilities and should be reviewed regularly to assess whether it has become more likely than not (probability has exceeded 50%).

Property damage by mob:

As Mob vandalized Astha's furniture and fixtures worth of BDT 75,000 which still remain within PPE, it should be removed and charged as expenses. Furthermore, management expects a lawsuit of BDT 2,000,000 which is more likely than not (i.e. with probability more than 50%). That means it should recognize a liability for this potential lawsuit with BDT 2,000,000. Astha should post following entries:

Misc. Loss	Dr.	75,000	
Property, Plant & Equipment	Cr.		75,000
Fines & Penalty Expenses	Dr.	2,000,000	
Provisions & Accruals	Cr.		2,000,000

Current tax expenses:

Astha recognized tax provision based on minimum tax liabilities. If any further tax is required based on profit before tax at 30% tax rate, provision will be required for incremental.

To calculate the profit before tax, adjusted profit before WPPF should be calculated on which WPPF expense should be calculated. After deducting WPPF expense from profit before WPPF, profit before tax would be determined.

Reported Profit before WPPF	1,040,479,222
Less: Revaluation gain	(853,625,465)
Less: Service Cost	(2,788,992)
Less: Finance Expenses	(1,289,985)
Less: Misc. Loss	(75,000)
Less: Fines & Penalty	(2,500,000)
Less: Professional Fees	(350,000)
Adjusted profit before WPPF	**179,849,780**
Less: WPPF Expenses (5/105)	(8,564,275)
Profit before Tax	**171,285,505**

Tax @ 30% = 51,385,651

As tax on profit before tax is lower than reported minimum tax of BDT 60,704,008, no further tax provision is required. However, for revied WPPF expenses, following entry will be required:

WPPF Expense	Dr.	8,564,275	
Provision for WPPF	Cr.		8,564,275

Deferred tax expenses:

Astha have considered all temporary differences for deferred tax except Gratuity obligations including OCI items and Revaluation gain and provision on lawsuits which raised from adjustment schedule. As gratuity obligation was not recognized earlier, its related deferred tax also should be considered.

Deferred tax on P&L items for 2024:

Source	**Carrying Amount**	**-**	**Tax Base**	**= DTD/(TTD)**
Lawsuit Exp provision	2,000,000	-	0	= 2,000,000
Gratuity Service Cost + Interest	4,078,977	-	0	= 4,078,977
Total Temporary Difference for P&L				= 6,078,977
Deferred Tax Income for profit or loss @ 30%				**= 1,823,693**

Deferred tax on OCI items for 2025:

Revaluation gain:

Revaluation Gain for the year	853,625,465
Less: Tax Base	0
Taxable/(deductible) temporary difference	**853,625,465**
Deferred tax liabilities/exp at appliable rate (5%)	**42,681,273**

Remeasurement loss:

Remeasurement loss for the year	(2,467,807)
Less: Tax Base	0
Taxable/(deductible) temporary difference	**(2,467,807)**
Deferred tax assets/income at appliable rate (30%)	**(740,342)**

In addition to above, prior year deferred tax adjustments for gratuity would be:

Deferred tax on remeasurement loss for 2024:

Remeasurement loss for the year	(254,652)
Less: Tax Base	0
Taxable/(deductible) temporary difference	**(254,652)**
Deferred tax assets at appliable rate (30%)	**(76,396)**

Deferred tax asset/liabilities from gratuity for 2024:

	2024	2023
Liabilities Carrying amount	(17,199,796)	(13,889,462)
Less: Tax base	0	0
Deductible temporary difference	**(17,199,796)**	**(13,889,462)**
Tax rate	30%	30%
Deferred Tax Assets	**5,159,939**	**4,166,839**

Deferred Tax income for the year 2024 = (5,159,939-4,166,839)	= 993,100
Less: Deferred tax income on re-measurement loss (OCI item)	= (76,396)
Deferred tax income for the year 2024 to be reported in P&L	= **916,704**

For incremental deferred tax balances following entry will be required:

To recognize Deferred tax on gratuity liability on 01 July 2023:

Deferred Tax Assets	Dr.	4,166,839	
Deferred Tax Income (R/E)	Cr.		4,166,839

To recognize Deferred tax on gratuity expenses on 30 June 2024:

Deferred Tax Assets	Dr.	993,100	
Deferred Tax Income (R/E)	Cr.		916,704
Re-measurement surplus (OCI)	Cr.		76,396

To recognize Deferred tax on gratuity expenses on 30 June 2025:

Deferred Tax Assets	Dr.	2,816,793	
Deferred Tax Income	Cr.		1,823,693
Re-measurement surplus (OCI)	Cr.		740,342
Revaluation Surplus	Dr.	42,681,273	
Deferred Tax Liabilities	Cr.		42,681,273

2.2. Conclusion

- Although Astha have not recognized gratuity obligation as per the Labor Act, 2006 in its earlier financial statements, it has now recognized the gratuity obligation through restating past financials. It is now showing annual costs in the income statement and reflecting long-term liabilities and related tax impacts properly.
- Land revaluation gains incorrectly treated as non-operating income instead of reporting in "Other Comprehensive Income". Because of the error revaluation gain was included in retained earnings instead of revaluation surplus in equity. By making appropriate adjustments, revaluation gain removed from non-operating income to other comprehensive income resulting in reduction of reported profits but make the accounts more transparent and compliant.
- Because profits were overstated earlier, the Workers' Profit Participation Fund (WPPF) has been recalculated after making all correction entries. This has lowered distributable earnings but ensured employees' share is fairly and accurately determined.
- Regulatory fines (BDT 500,000) and professional fees (BDT 350,000) for staff credential checks were missed in the draft accounts. These should now be recorded. Meanwhile, no provision is yet required for potential lawsuits for BDT 1,500,000 due to doctor's malpractice. However, it should be disclosed as contingent risks.
- The mob-related property damage (BDT 75,000) must be written off. Furthermore, a lawsuit of BDT 2,000,000 for erroneous treatment is highly probable, it should be provided for. Recognizing these ensured liabilities aren't understated.
- After all adjustments, profit before tax falls to about BDT 171 million. Even though this lowers profitability, no extra tax provision is needed because the company has already provided for the minimum tax.
- Although there is no impact on the current tax, impacts on deferred tax are significant. Due to gratuity obligations and lawsuit provisions adjustments, deferred tax income and deferred tax asset of BDT 1,823,693 have been generated. However, significant amount of deferred tax liability for BDT 42.7 million arises on land revaluation gain, while a deferred tax liabilities of BDT 740,342 has been generated on current year actuarial re-measurement loss.

- Although these adjustments reduce reported profits and increase liabilities, they also improve transparency. Equity will now include a new re-measurement reserve and an increment in the existing revaluation reserve, giving a clearer picture of financial health and strengthening compliance with accounting standards.

2.3. Recommendation

- Establish a formal gratuity recognition process with regular actuarial valuations to ensure liabilities are updated annually and reflected accurately in both financial statements and tax planning.
- Strengthen accounting policies for performing revaluations on regular interval, ensuring all future land and asset revaluations are correctly routed through "Other Comprehensive Income" and reflected in the appropriate equity reserves.
- Enhance compliance monitoring by proactively identifying regulatory requirements (such as credential verification of doctors) to avoid fines, penalties, and unrecorded expenses in future reporting periods.
- Improve risk management processes by maintaining an updated litigation and claims register, with timely assessment of contingencies versus provisions, to ensure lawsuits and damages are reflected properly in the accounts.
- Introduce a property and asset safeguarding mechanism (e.g., insurance coverage and improved physical security) to minimize financial loss from vandalism or external disturbances.
- Align tax reporting more closely with accounting adjustments, ensuring that minimum tax provisions, deferred tax assets, and liabilities are reconciled on a timely basis to avoid sudden adjustments.
- Build equity transparency by maintaining clear and separate reserves for revaluation surplus and remeasurement reserves, which will provide stakeholders with a more accurate picture of retained earnings versus other comprehensive items.
- Strengthen financial reporting controls through periodic internal audits and management reviews, so errors like misclassification of gains, unrecorded liabilities, and missed disclosures are identified early.

3. Evaluation of adjusted financial statements considering non- financial info (Requirement-a-ii)

3.1. Financial Evaluation of Astha after financial statements after adjustment

Astha's financial statement has been adjusted in Appendix-1 with the accounting adjustments mentioned above. Analysis has been performed on the adjusted financial statements.

Revenue:

Astha's post adjustment revenue for 2025 stands at BDT 3,980,671,132 which has grown significantly by 13.2% (BDT 464,197,041) from prior year revenue of BDT 3,516,474,091. Although Astha achieved a healthy growth, the overall healthcare sector in Bangladesh recorded a CAGR of 15–20% in recent years. Compared to industry growth, Astha's performance is moderate, reflecting both expansion in core services and limitations in service diversification.

Growth has been driven mainly by medical services which grow by 18.6% and contributes 46.2% of total revenue. Diagnosis services which contribute around 25% in total revenue has grown by 11%

which also contributed to revenue growth for the year. Finally with pharmacy revenue that contributes around 28.0% of total revenue has increased by 7.2%. The balanced mix provides stability in Astha's revenue earning.

Cost of Revenue:

Post adjustment Cost of Revenue of 2025 has been derived at BDT 1,614,813,299 which has been increased by 11.1% (BDT 161,558,933) from prior year cost of revenue of BDT 1,453,254,366 matching revenue growth. Cost of revenue has increased as revenue as increased. However, cost of revenue growth is lower than revenue growth probably due to management efficiency in cost reduction.

During the year, medical services revenue achieved 14.7% growth whereas Diagnostic Services achieved 12% and Pharmacy achieved 9.3% growth. Contrary to highest revenue contribution, medical services generate lowest percentage (21.9%) of Cost of Revenue. Whereas pharmacy has the smallest contribution in revenue generation but accounts for largest portion (52.6%) of cost of revenue. Diagnostic Services generates remaining 25.8% of cost of revenue and inline with revenue generation contribution. Overall, Astha has maintained excellent controlled growth in cost of revenue despite high inflation and increasing material costs.

GP and GP Margin:

Astha's adjusted gross profit for the year has been derived at BDT 2,365,857,833, which has increased by BDT 302,638,108 from the prior year's gross profit of BDT 2,063,219,725. This represents a 14.7% growth in gross profit, which is consistent with the 13.2% growth in revenue during the year. The increase in gross profit demonstrates a modest improvement in operational performance, especially in the context of political unrest and unstable business conditions that prevailed during FY2025. The growth is mainly attributable to the hospital's increased patient inflow, higher utilization of diagnostic services, and steady performance of the pharmacy segment.

Despite rising medical consumable and utility costs, Astha was able to maintain its profitability through improved cost control and optimized resource utilization. The growth in gross profit, while moderate compared to revenue increase, is still a commendable performance considering the adverse healthcare market conditions in Bangladesh during the year.

During the year, Astha's gross profit margin improved slightly to 59.43% from 58.70% in the prior year. This relative stability in gross margin reflects effective management of direct costs despite sharp increases in procurement and freight expenses due to inflation and political disruption. The marginal improvement in GP margin is driven by better pricing strategy in diagnostic services and higher contribution from inpatient and pharmacy operations.

Overall, the consistent gross profit growth and stable margin position indicate a strong operational base for Astha General Hospital Limited, showing resilience in a volatile economic and political environment. Sustaining this efficiency and focusing on cost optimization will be key to enhancing profitability in the coming years.

Net Profit and Net Profit Margin

Astha initially reported a Net Profit of BDT 941,316,632. However, after making all accounting, legal, and compliance adjustments (including exclusion of non-recurring revaluation gain, recognition of gratuity liability, and probable legal penalties), Astha reported an adjusted net income of BDT 123,493,238, which represents a sharp increase of BDT 119,886,795 compared to the prior year's

adjusted net profit of BDT 3,606,443. This is a remarkable 3,324% growth in net profit, primarily driven by higher operational efficiency and improved revenue performance across hospital, diagnostic, and pharmacy services.

Despite the substantial growth, overall profitability remains modest. During FY2025, Astha's net profit margin improved massively to 3.10% from 0.10% in FY2024, reflecting a rebound from a weak prior-year performance. The improvement indicates better control over administrative and financing costs, as well as a recovery in patient and diagnostic volumes after the disruptions of the previous year.

Although the profit growth appears impressive, it should be interpreted cautiously as the prior year's profit base was unusually low due to one-off expenses. Going forward, Astha needs to sustain this momentum by focusing on cost efficiency, service diversification, and maintaining financial discipline amid a volatile economic and political environment.

Current and Quick Ratio

Financial analysis shows that Astha's current ratio for the year 2025 is 1.39, compared to 1.48 in the prior year. This indicates a slight decline in the ratio, suggesting that the company's short-term liquidity position has weakened during the year. The current ratio measures the hospital's ability to meet its short-term obligations using its current assets. Although a ratio above 1 generally indicates that Astha has sufficient current assets to cover its current liabilities, the declining trend reflects a tightening of working capital. This reduction may be linked to increased operating expenses, higher payables, and greater short-term funding requirements driven by expansion and inflationary pressures in procurement of medical supplies. The fall in current ratio implies that Astha's working capital cushion has narrowed, potentially due to slower recovery of receivables and increased dependence on internal cash flows rather than external financing.

Quick ratio represents Astha's ability to meet current liabilities using its most liquid current assets, excluding inventories. From the financial statements, Astha's quick ratio for 2025 stands at 1.14, compared to 1.18 in the previous year, indicating a slight deterioration in immediate liquidity. The drop in quick ratio suggests that the proportion of cash and receivables relative to current liabilities has decreased, mainly due to higher consumption of liquid funds for operational expansion and replacement of damaged assets during the year. Although the ratio remains above 1, showing that Astha can meet short-term obligations without selling inventory, the declining trend points to reduced liquidity flexibility. To maintain a strong financial position, Astha should focus on improving receivable collections, optimizing inventory levels, and managing payables efficiently to preserve liquidity stability.

Debt to Equity and TIE Ratio

The debt-to-equity ratio indicates the extent to which Astha's operations are financed through debt compared to shareholders' equity. Based on the adjusted financial statements, Astha's debt-to-equity ratio for 2025 stands at 0.02, compared to 0.04 in the previous year. This marks a notable improvement in capital structure, showing that the company has reduced its reliance on external debt financing. The reduction primarily stems from repayment of long-term borrowings and a significant increase in retained earnings during the year. The low debt ratio reflects that over 98% of Astha's capital structure is financed through equity, indicating a very conservative financial position. While this minimizes financial risk and interest burden, it may also suggest that Astha is under-leveraged and potentially missing out on opportunities to use inexpensive debt to fund expansion or modernization projects. Astha may consider increase gearing in the equity structure what may increase return to the shareholders.

The Times Interest Earned (TIE) ratio, which measures the company's ability to meet its interest obligations using its operating profits, stands at 3.93 in 2025, compared to 1.46 in 2024. This represents a significant improvement in Astha's capacity to service debt. The increase in TIE ratio is primarily due to a substantial rise in operating profit, supported by higher revenue and better cost control, while finance costs have declined marginally during the year. The current ratio indicates that Astha earns nearly four times its annual interest expense from operations, suggesting a strong debt-servicing position. The improvement demonstrates enhanced financial stability and better profitability management. However, maintaining this level will depend on Astha's ability to sustain operating efficiency and manage any future financing decisions prudently as it considers expansion into new markets.

Asset Turnover and Return on Assets

The asset turnover ratio measures how efficiently Astha utilizes its total assets to generate revenue. For the year 2025, Astha's asset turnover ratio stands at 0.31, which is consistent with 0.31 in 2024, showing no significant change in operational efficiency year over year. This stability indicates that Astha's revenue growth has kept pace with the increase in its total asset base. Despite the hospital's expansion in facilities and infrastructure upgrades, these investments are yet to yield proportionate revenue benefits. As newly added facilities reach full operational capacity in the coming periods, the asset utilization rate is expected to improve. Therefore, the current stability in asset turnover reflects a transitional phase, where recent capital investments are still ramping up toward optimal productivity.

The Return on Assets (ROA) ratio, which assesses how effectively Astha converts its total assets into net earnings, stands at 0.97% in 2025, a sharp improvement from 0.03% in 2024. This substantial increase in ROA is primarily driven by a significant rise in adjusted net profit following operational recovery and improved cost management. However, despite the strong rebound, the overall ROA remains relatively low, suggesting that a large portion of Astha's asset base, particularly in property, plant, and medical equipment, is underutilized in generating income. The low return is characteristic of capital-intensive healthcare operations, but to strengthen efficiency, Astha should focus on increasing patient turnover, optimizing asset use across departments, and leveraging its existing infrastructure more effectively.

Return on Equity

Astha General Hospital Limited's Return on Equity (ROE) for the year 2025 stands at 1.12%, compared to 0.04% in 2024. This represents a significant improvement in shareholders' return, primarily driven by the strong recovery in net profit during the year. The rise in ROE reflects Astha's improved profitability and effective cost management following a challenging prior year, despite the ongoing political instability and increased cost of operations in the healthcare sector.

The improvement in ROE was largely supported by higher patient inflow, increased diagnostic and pharmacy sales, and better control over administrative and financial expenses. However, while the increase is encouraging, the overall return remains relatively low for a capital-intensive healthcare organization, indicating that substantial investments in fixed assets and infrastructure are yet to translate into proportionate shareholder returns. An ROE of 1.12% suggests that Astha is generating modest earnings from its equity base, which exceeds BDT 11 billion. To further enhance shareholder value, the company should focus on improving asset utilization, optimizing operating efficiency, and strategically leveraging its strong equity position to fund higher-return projects. Although current performance shows recovery momentum, sustained improvement in ROE will depend on the successful execution of expansion plans and consistent profitability growth in the coming years.

3.2. Evaluation of non-financial performance

Astha General Hospital Limited has evolved into one of Bangladesh's most trusted private healthcare providers, known for its commitment to quality patient care, advanced diagnostic capabilities, and ethical medical practices. While the hospital has made strong progress in service expansion, technology adoption, and community engagement, it faces several non-financial challenges relating to operational efficiency, environmental management, workforce stability, patient safety, and regulatory compliance.

Operational Efficiency

Astha has expanded its operations significantly, with its flagship hospital in Dhaka and plans to extend to Chattogram and Sylhet. Its integrated Hospital Management System (HMS), centralized procurement of medical supplies, and 24/7 emergency services reflect strong operational structure. The introduction of telemedicine, digital health records, and patient identification through "Astha Card" demonstrates increasing process automation and digital transformation. However, challenges persist in resource utilization and patient flow management. Occasional overstay of patients in ICU or private wards, fragmented communication during peak hours, and supply chain inefficiencies in medical consumables have affected service consistency. Moreover, operational disruptions during political unrest and staff shortages in critical care units underscore the need for better contingency planning and internal coordination.

Environmental Sustainability

Astha has adopted several responsible environmental practices, such as waste segregation, biomedical waste disposal through certified vendors, and regular safety audits to ensure compliance with health and environmental regulations. It also enforces strict infection control and maintains high sanitation standards. Nevertheless, the healthcare industry's environmental challenges remain evident at Astha. Energy consumption from diagnostic and imaging equipment, inadequate wastewater treatment, and reliance on fossil-fuel-based electricity contribute to its environmental footprint. Although management has acknowledged these issues, investment in sustainable energy systems such as solar integration or water treatment facilities remains limited. A long-term sustainability framework is needed to align Astha with evolving ESG standards in the healthcare sector.

Employee & Labor Relations

Astha employs a large multidisciplinary workforce including doctors, nurses, technicians, and administrative staff. The HR department has implemented continuous medical education, life support training (BLS and ACLS), and performance-based incentives to enhance employee competence and motivation. However, high turnover among nursing and junior administrative staff continues to pose operational and quality risks. Instances of staff fatigue and communication lapses during emergencies have been noted, especially in the emergency and diagnostic departments. Although the hospital provides competitive pay and welfare programs, ensuring consistent skill development, maintaining fair workload distribution, and strengthening staff retention remain ongoing priorities.

Market Positioning & Brand Strength

Astha enjoys a strong and trusted reputation among middle and upper-middle income patients due to its advanced diagnostic facilities, ethical medical practices, and patient-centric care. Its growing presence in telemedicine and preventive healthcare programs has enhanced its accessibility and brand visibility. However, rising competition from new private hospitals and specialized clinics, coupled with reputation risk from isolated patient safety incidents (such as the medication error case reported in early 2025), has challenged public perception.

Corporate Governance & Compliance

Astha's governance framework shows notable strengths, including IFRS-compliant financial reporting, presence of qualified finance professionals, and a functional internal control system. The recent appointment of a new audit firm reflects management's effort to strengthen governance. However, the hospital has previously faced audit qualifications due to non-recognition of employee benefit obligations, highlighting gaps in compliance oversight. Moreover, related-party concerns, e.g. such as the personal connection between the Finance Director and the current auditor, pose potential independence risks. Strengthening audit committee functions, ensuring regulatory compliance with FRC standards, and improving transparency will be critical to sustaining governance integrity.

Corporate Social Responsibility (CSR) & Community Impact

Astha has demonstrated a strong sense of social responsibility through free medical camps, vaccination drives, health awareness programs, and disaster relief initiatives. It actively supports community health, education, and blood donation campaigns. These programs align well with its mission to promote accessible healthcare for all. However, Astha's CSR reporting remains informal and narrative-driven, with limited quantifiable outcome measures. To enhance credibility and long-term social impact, the hospital should develop structured CSR metrics, publish periodic impact reports, and engage third-party evaluators. Strengthening CSR governance will help position Astha as a socially responsible and ethically governed healthcare institution in Bangladesh.

3.3. Conclusion

- Astha's revenue grew by 13.2% in 2025, reflecting solid recovery but still lagging the industry's 15–20% growth rate. The expansion in medical and diagnostic services drove performance, though diversification remains limited.
- Cost of revenue increased by 11.1%, slightly below revenue growth, showing efficient cost management amid inflation and political disruptions which helped to sustain its profitability.
- Gross profit rose by 14.7% to BDT 2,365.86 million, with margin improving to 59.43%. This highlights effective cost control and consistent operational performance despite market volatility.
- Adjusted net profit jumped to BDT 123.49 million from BDT 3.61 million, with margin up to 3.10%. Profit rebound reflects recovery from prior-year setbacks, though overall modest remain.
- Current and quick ratios declined to 1.39 and 1.14, indicating slightly tighter liquidity. The hospital still maintains adequate short-term solvency but faces reduced cash flexibility.
- Debt-to-equity ratio fell to 0.02, showing minimal reliance on debt and low financial risk. However, under-leveraging may limit growth opportunities and shareholder returns.
- Times Interest Earned improved sharply to 3.93 from 1.46, reflecting stronger operating profit and a healthier ability to cover interest obligations.
- The asset turnover ratio remained steady at 0.31, showing consistent asset utilization. Newly added facilities are expected to enhance efficiency as they reach full operational capacity.
- Return on Assets rose to 0.97% and Return on Equity to 1.12%, reflecting profitability recovery. However, overall returns are still low due to heavy asset investment and limited yield.
- Strong digital integration and expansion drive efficiency, but patient flow issues, supply delays, and staffing gaps still constrain peak performance.
- Compliance in waste management is maintained, but limited investment in green technologies leaves room for improvement in ESG performance.

- Training and incentive programs strengthen workforce quality, though high turnover and workload imbalance continue to challenge service consistency.
- Astha remains a trusted healthcare brand but must reinforce clinical governance and service quality to protect its reputation amid rising competition.
- IFRS compliance and professional oversight are strengths, but internal control gaps and audit independence risks require closer governance attention.
- Astha's community health programs demonstrate strong social commitment. Structured CSR reporting and third-party evaluation would enhance accountability and credibility.

3.4. Recommendation

- Strengthen cost control and procurement efficiency by implementing centralized monitoring, reducing material wastage, and renegotiating supplier terms to sustain profitability amid rising operating costs.
- Diversify revenue streams by expanding specialized medical services, launching advanced diagnostic packages, and scaling telemedicine operations to capture new market opportunities and enhance patient reach.
- Improve cash flow and liquidity management through faster receivable collection, optimized working capital utilization, and prudent short-term financing to maintain operational stability.
- Enhance asset utilization by increasing patient throughput, optimizing bed occupancy rates, and ensuring timely operational readiness of new facilities to generate stronger returns on investment.
- Invest in sustainable energy solutions and efficient waste management systems to minimize environmental impact and align with healthcare sector ESG standards.
- Standardize employee training, strengthen retention programs, and create structured career development pathways to reduce turnover and ensure consistent quality of patient care.
- Reinforce governance and transparency by ensuring auditor independence, expanding internal audit coverage, and improving board-level oversight of compliance and financial reporting.
- Formalize CSR initiatives with measurable outcomes, periodic external evaluations, and transparent reporting to strengthen community trust and demonstrate social accountability.

4. Evaluate of the Strengths, Weaknesses, Opportunities, and Threats (SWOT) of Astha's existing business (Requirement b-i):

4.1. SWOT Analysis:

Strengths

- ***Trusted Healthcare Brand with Expanding Footprint***
  Astha General Hospital Limited has built a strong reputation as a trusted private healthcare provider in Bangladesh, recognized for its patient-centric approach, ethical medical practices, and quality care delivery. With its flagship facility in Dhaka and expansion plans into Chattogram and Sylhet, Astha is strategically positioned to serve a wider patient base and strengthen its presence across key regional markets.

- ***Comprehensive and Diversified Service Portfolio***
  Astha offers an extensive range of healthcare services, including inpatient and outpatient care, emergency treatment, advanced diagnostics, and a fully integrated pharmacy. This diversified service portfolio ensures steady revenue streams, minimizes risk from sectoral fluctuations, and reinforces Astha's role as a full-service medical institution catering to diverse patient needs.

- ***Advanced Digital Infrastructure and Technological Capability***
  Astha's investment in digital transformation, through its integrated Hospital Management System (HMS), electronic medical records, and telemedicine services, has significantly enhanced efficiency, coordination, and service accessibility. This forward-looking approach positions the hospital as a modern, technology-driven healthcare provider ready for the next phase of digital health evolution.

- ***Financial Stability and Strong Capital Structure***
  Astha demonstrates solid financial health, with a low debt-to-equity ratio of 0.02 and consistent revenue growth of 13.2% in 2025. Its conservative capital structure, strong interest coverage, and growing profit margin reflect prudent financial management, enabling sustainable operations and self-funded expansion capacity.

- ***Skilled and Committed Workforce***
  Astha's multidisciplinary team of doctors, nurses, and healthcare professionals remains one of its core strengths. Continuous professional development, structured training, and performance-based incentives contribute to service excellence, operational efficiency, and improved patient outcomes.

- ***Strong Governance and Financial Transparency***
  Astha upholds sound corporate governance, maintaining IFRS-compliant reporting, regular external audits, and a qualified leadership team with diverse expertise. Transparent financial management and internal control systems reinforce investor confidence and institutional credibility.

- ***Commitment to CSR and Community Health Initiatives***
  Astha's commitment to social responsibility is reflected through health awareness programs, vaccination campaigns, free medical camps, and disaster response initiatives. These efforts enhance public goodwill, strengthen community relations, and align with the organization's mission to make healthcare accessible and ethical for all.

Weaknesses

- ***Weak Internal Controls and Financial Reporting Gaps***
  Several post-closing adjustments were required in Astha's 2025 financial statements, highlighting weaknesses in financial reporting accuracy and internal control systems. These weaknesses can undermine transparency and stakeholder confidence, particularly for a healthcare organization of public interest.

- ***Limited-Service Diversification Beyond Core Operations***
  Astha's revenue continues to rely heavily on inpatient and diagnostic services, while high-value specialized care areas such as advanced surgery, fertility, and oncology remain underdeveloped. This limited diversification restricts its ability to tap into fast-growing healthcare segments and exposes the hospital to concentration risk within its existing service portfolio.

- ***Underutilization of Digital and Telehealth Platforms***
  Although Astha has implemented an integrated Hospital Management System and telemedicine features, its digital health strategy remains underdeveloped. Limited online appointment systems, patient apps, and digital engagement tools reduce operational efficiency and restrict access for remote patients. These are areas of growing importance in Bangladesh's evolving healthcare landscape.

- ***Operational Inefficiencies and Inconsistent Service Standards***
  New and expanding hospital units face challenges in maintaining consistent quality, staff training, and patient flow management. Operational bottlenecks, such as delays in diagnostics, overcrowded outpatient departments, and uneven resource allocation etc., undermine service efficiency and patient satisfaction.

- ***Low Return on Investment and Underutilized Assets***
  Despite substantial investment in infrastructure and equipment, asset utilization remains modest, with low ROA (0.97%) and ROE (1.12%). This suggests that new facilities and high-value medical equipment have yet to achieve optimal productivity, limiting the hospital's overall profitability and financial leverage potential.

- ***Workforce Retention and Fatigue Concerns***
  High turnover among nursing and junior administrative staff continues to strain operations, increasing recruitment costs and disrupting service continuity. Additionally, staff fatigue in emergency and critical care units has been observed, posing both quality and safety risks in patient care delivery.

- ***Inadequate Environmental and Sustainability Practices***
  While Astha complies with biomedical waste management standards, it lacks comprehensive sustainability initiatives such as renewable energy integration, water recycling, and carbon footprint reduction. The absence of a long-term environmental policy exposes the hospital to reputational and regulatory risks as ESG standards tighten.

- ***Governance and Independence Risks***
  Although Astha has qualified professionals overseeing its finance and audit functions, the auditor's non- enlistment with FRC and familial connection with senior management raise questions about independence and governance transparency. Weak audit committee oversight may hinder early detection of financial or compliance irregularities, potentially affecting institutional credibility.

Opportunities

- ***Expanding Reach Through Regional Hospital Networks and Franchising Models***
  Astha can accelerate its expansion by establishing satellite hospitals, diagnostic centres, or franchise-operated clinics in semi-urban and rural regions. Partnering with local healthcare entrepreneurs would enable Astha to extend quality medical services to underserved communities while minimizing capital expenditure and operational strain on the core hospital.

- ***Capitalizing on Rapid Growth in Bangladesh's Healthcare Sector***
  Bangladesh's private healthcare market continues to grow at over 15% annually, driven by rising income levels, urbanization, and increasing demand for quality medical services. Astha is well-positioned to capture a greater market share by expanding into emerging medical hubs

such as Chattogram, Sylhet, and Khulna, where demand for modern healthcare infrastructure is accelerating.

- ***Investing in Medical Technology and Process Automation***
  With rising operational costs and growing patient volumes, automation and digital tools can substantially improve efficiency. Introducing AI-based diagnostic systems, automated patient registration, and digital treatment tracking can reduce human error, enhance productivity, and elevate the overall patient experience.

- ***Scaling Up Digital Health and Telemedicine Services***
  As digital health adoption rises in Bangladesh, Astha can strengthen its telemedicine platform, develop a patient-focused mobile app, and integrate online appointment booking, digital prescriptions, and remote consultation services. Expanding digital engagement will help Astha reach remote populations and attract tech-savvy patients seeking convenience and accessibility.

- ***Penetrating Underserved Rural and Tier-2 Healthcare Markets***
  With limited access to quality healthcare in smaller towns and rural areas, Astha can introduce smaller-format hospitals, diagnostic satellite centres, or community clinics. This strategy would not only address the healthcare gap but also strengthen Astha's social impact and national brand presence.

- ***Unlocking Growth Capital and Visibility Through an IPO***
  Astha may consider pursuing an Initial Public Offering (IPO) to raise expansion capital, diversify funding sources, and reduce reliance on internal cash flows. A public listing would enhance transparency, corporate governance, and investor confidence, positioning Astha as a leading, professionally managed healthcare organization in Bangladesh.

- ***Strengthening Partnerships with Government and NGOs***
  Collaborating with government agencies, NGOs, and international health organizations can help Astha participate in national health programs such as vaccination drives, maternal care, and public health education. Such partnerships would enhance brand credibility and contribute to long-term sustainable growth.

- ***Introducing Specialized Centres of Excellence***
  Astha can strengthen its competitive edge by establishing specialized units in high-demand areas such as cardiology, oncology, fertility, and critical care. Focused investment in these disciplines would attract a broader patient base, improve profitability, and establish Astha as a regional leader in specialized healthcare services.

Threats

- ***Economic Instability and Reduced Healthcare Affordability***
  High inflation, currency depreciation, and rising living costs are reducing disposable income for many households, limiting their ability to afford private healthcare. This economic pressure may shift patients toward lower-cost clinics or government hospitals, affecting Astha's revenue growth and service utilization rates.

- ***Rising Operating Costs and Workforce Challenges***
  Sharp increases in utility, medical consumable, and maintenance costs continue to squeeze margins. Simultaneously, nursing shortages, wage pressure, and staff fatigue are straining operations and service delivery, posing a risk to patient satisfaction and overall efficiency.

- ***Regulatory and Compliance Risks in the Healthcare Sector***
  Frequent changes in healthcare regulations, taxation, and licensing requirements create uncertainty for long-term planning. Stricter enforcement by regulatory bodies such as the Directorate General of Health Services (DGHS) may expose gaps in compliance and increase administrative costs.

- ***Supply Chain Disruptions and Import Dependency***
  Political unrest, infrastructure limitations, and reliance on imported medical equipment and consumables have made Astha's supply chain increasingly fragile. Disruptions in global logistics or currency fluctuations can delay critical supplies, affecting hospital operations and patient care continuity.

- ***Intensifying Competition from Emerging Healthcare Providers***
  New entrants in private healthcare—especially specialized hospitals and diagnostic chains—are capturing market share through aggressive pricing and service innovation. This growing competition could dilute Astha's market presence, particularly in key urban centres.

- ***Growing Scrutiny on Service Quality and Patient Safety***
  With increasing public awareness and media focus on healthcare standards, any lapse in service quality, medical error, or ethical misconduct could escalate into major reputational damage. Maintaining consistent patient safety protocols is critical to preserving trust.

- ***Rapid Technological Advancements and Digital Lag Risks***
  Global competitors are rapidly adopting AI-based diagnostics, robotic surgery, and data-driven clinical systems. Without timely upgrades to its technology infrastructure, Astha risks falling behind in efficiency, patient engagement, and operational accuracy, reducing its competitiveness in a fast-evolving healthcare market.

4.2. Conclusion:

- Astha General Hospital Limited has strengthened its position as a trusted and ethical private healthcare provider in Bangladesh, with a growing presence and strong patient confidence.
- Revenue grew by 13.2% in 2025, supported by balanced contributions from medical, diagnostic, and pharmacy services, reflecting resilience amid economic and political challenges.
- Gross profit increased by 14.7% and the margin improved to 59.43%, showing effective cost control and efficient resource utilization despite inflationary pressures.
- Net profit rose sharply to BDT 123.49 million, with the net margin improving to 3.10%, marking a significant financial turnaround from the previous year.
- Liquidity slightly tightened, with current and quick ratios declining to 1.39 and 1.14, indicating the need for stronger working capital and cash flow management.
- Low financial leverage (debt-to-equity ratio of 0.02) demonstrates financial prudence and stability, although moderate use of debt could accelerate future expansion.
- Governance practices have improved, but weaknesses in internal control, year-end adjustments, and auditor independence require immediate corrective action.
- Operational challenges persist, including uneven staff distribution, delayed coordination, and inefficiencies in newly expanded facilities that affect service quality.

- High employee turnover and staff fatigue continue to impact operational consistency; stronger retention and structured training programs are necessary.
- Limited investment in sustainability and energy-efficient technologies leaves room for improvement in environmental and ESG performance.
- CSR and community health initiatives, such as free medical camps and vaccination programs, enhance Astha's public image and social contribution.
- Bangladesh's expanding healthcare sector offers significant growth opportunities, and Astha can leverage its brand strength, digital potential, and strong equity base to expand into new regions and specialized care services.

4.3. Recommendation:

- Strengthen internal controls and financial reporting to ensure transparency, compliance, and reliability by automating accounting processes and maintaining auditor independence.
- Scale up digital healthcare services by developing a unified patient app with telemedicine, online appointment booking, digital billing, and integrated electronic health records.
- Invest in automation and technology upgrades such as AI-assisted diagnostics, automated scheduling, and smart data systems to enhance efficiency and reduce operational errors.
- Improve workforce retention and performance through structured training, clear career development paths, and incentive programs that strengthen employee commitment and service quality.
- Expand regionally through low-capital models like franchise-operated diagnostic centres and satellite clinics to reach underserved semi-urban and rural markets efficiently.
- Adopt sustainability and ESG practices by introducing solar energy systems, efficient waste management, and environmentally responsible infrastructure improvements.
- Enhance governance and board oversight by establishing an independent audit committee, tightening compliance monitoring, and improving financial accountability.
- Diversify funding sources by leveraging Astha's strong equity base and exploring options such as strategic partnerships or an IPO to support long-term expansion and modernization.

5. NPV analysis of the investment options, recommendation with justification and comment on compatibility with vision and mission (Requirement b-ii):

Board is currently considering two investment options which are mutually exclusive. First option is to Acquire Fair Hospital Limited, and second option is to set up Astha Healthcare Network Limited. Options are mutually exclusive, and the board of directors plans to decide based on the NPV of the options and compatibility of Astha's mission & vision. To perform NPV analysis, the board prefers the discount rate to be determined through CAPM formula where CAPM rate = Risk free rate + market beta (Market rate – Risk Free rate). Astha management has considered coupon rate of 10-year government bond as risk free rate.

According to the provided information, Risk Free rate is 10.5%; Market Return 12.5%, and market beta is 1.25. Therefore, the CAPM rate = 10.5% + 1.25(12.5%-10.5%) = 10.5% + (1.25 x 2%) = 10.5% + 2.5% = 13% which will be used as discount rate in NPV analysis.

5.1. NPV Analysis of "Acquire Fair Hospital Limited":

Astha is considering acquiring Acquire Fair Hospital Limited at Chattogram to expand its operation to other parts of Dhaka and to other major cities. Current owner of Fair Hospital has expressed interest in selling the hospital share a 5-year financial forecast which Astha has modified to

Fair finance team forecasted a revenue of BDT 995,000,000 in 2026 which Astha expects after acquisition to consistently increase at 12% per Anum. Astha Also expect to decrease direct expenses to 40% of revenue from 45% forecasted by Fair's management. Operating expenses are expected to be 35% of revenue in 2026 but increase by 10% annually till 2030. Astha's board plans to offer BDT 4,000,000,000 to Fair's current owner for as compensation. Astha's Board estimates that it would need to spend BDT 50,000,000 in 2026 and BDT 20,000,000 in 2027 for rebranding and renovation of the facility. Free cash flows are expected to increase at 5% after the year 2030.

Based on the provided forecast, following cash flows can be draws up for NPV calculation:

Particulars	**2025**	**2026**	**2027**	**2028**	**2029**	**2030**
Capital Expenditure	(4,000,000,000)	-	-	-	-	-
Revenue	-	995,000,000	1,114,400,000	1,248,128,000	1,397,903,360	1,565,651,763
Direct Expenses	-	(398,000,000)	(445,760,000)	(499,251,200)	(559,161,344)	(626,260,705)
Operating Expenses	-	(348,250,000)	(383,075,000)	(421,382,500)	(463,520,750)	(509,872,825)
Rebranding Exp.	-	(50,000,000)	(20,000,000)	-	-	-
Free Cash Flow	**(4,000,000,000)**	**198,750,000**	**265,565,000**	**327,494,300**	**375,221,266**	**429,518,233**
Terminal Value	-	-	-	-	-	5,637,426,807
Total Cash flow	**(4,000,000,000)**	**198,750,000**	**265,565,000**	**327,494,300**	**375,221,266**	**6,066,945,040**
PV Factor	1	0.8850	0.7831	0.6931	0.6133	0.5428
Present Value	**(4,000,000,000)**	**175,893,750**	**207,963,952**	**226,986,299**	**230,123,202**	**3,293,137,768**

NPV	**134,104,971**

* Terminal value = Cash flow @2030/(d-g) where d is discount rate of 13% & g is growth rate of 5%.
= (429,518,233 x 1.05)/ (13%-5%)
= 5,637,426,807

Based on the calculation, it is expected that acquisition of Fair Hospital Limited will generate NPV of BDT 134,104,971. As NPV of the option is positive, Astha may proceed with acquisition of Acquire Fair Hospital Limited.

5.2. NPV Analysis of "Astha Healthcare Network Limited":

Second option at the hand of Astha board is to launch "Astha Healthcare Network" bringing hospitals clinics, and diagnostic centres under its umbrella as network members. According to the plan, any healthcare providers can join as "Astha Healthcare Network Members," gaining brand, patient, and system benefits for an annual fee. In return, Astha ensures quality monitoring, shared medical data access, and integrated service systems across facilities.

Astha plans to set up "Astha Healthcare Network Limited" as fully owned subsidiary by investing BDT 500,000,000 as Share capital. It expects to charge annual membership fee of BDT 1,500,000 for small sized facilities and BDT 2,500,000 for Mid-sized members. It expects to take 20 small sized and 10 mid-sized facilities as members in the first year and increase membership by 15 small and 10 medium facilities in each of the next 3 years when membership will reach maximum capacity. Once membership reaches maximum capacity, annual membership fees are expected to increase 10% thereafter. It is estimated that direct expenses and operating expenses are to be 50% and 20% of the membership fees respectively. Cash flow after 5th year is likely to grow at 4% perpetually.

From this following cash flow can be drawn for NPV analysis:

Particulars	**Year 0**	**Year 1**	**Year 2**	**Year 3**	**Year 4**	**Year 5**
Capital Expenditure	(500,000,000)	-	-	-	-	-
Fee from small facility	-	30,000,000	52,500,000	75,000,000	97,500,000	107,250,000
Fee from medium facility	-	37,500,000	62,500,000	87,500,000	112,500,000	123,750,000
Total Fee income	-	**67,500,000**	**115,000,000**	**162,500,000**	**210,000,000**	**231,000,000**
Direct Expenses (50%)	-	(33,750,000)	(57,500,000)	(81,250,000)	(105,000,000)	(115,500,000)
Operating Exp (20%)	-	(13,500,000)	(23,000,000)	(32,500,000)	(42,000,000)	(46,200,000)
Free Cash Flow	**(500,000,000)**	**20,250,000**	**34,500,000**	**48,750,000**	**63,000,000**	**69,300,000**
Terminal Value	-	-	-	-	-	800,800,000
Total Cash flow	**(500,000,000)**	**20,250,000**	**34,500,000**	**48,750,000**	**63,000,000**	**870,100,000**
Present Value Factor	1	0.8850	0.7831	0.6931	0.6133	0.5428
Present Value	**(500,000,000)**	**17,921,250**	**27,016,950**	**33,788,625**	**38,637,900**	**472,290,280**

NPV	**89,655,005**

* Terminal value = Cash flow @ Y6/(d-g) where d is discount rate of 13% & g is growth rate of 4%.
= (69,300,000 x 1.04)/ (13%-4%)
= 800,800,000

Based on the calculation, setting up "Astha Healthcare Network Limited" will generate a negative NPV of BDT 89,655,005. As NPV of the option is positive, it will be worthwhile to invest in setting up "Astha Healthcare Network Limited".

5.3. Execution recommendation to the board with justification:

Astha's board has two mutually exclusive options available at their hand for execution. Therefore, it should select the option that is commercially viable, provides highest NPV and supports its mission and vision. The NPV analysis of the investment options reveals that acquisition of Fair Hospital Limited will generate NPV of BDT 134,104,971 and setting up "Astha Healthcare Network Limited" will generate a negative NPV of BDT 89,655,005. Although both options have positive NPV, but acquisition of Fair Hospital Limited will generate highest NPV. Therefore, acquiring Fair Hospital Limited is more commercially viable and Astha should select this investment option.

5.4. Comment on compatibility on investment options with Astha's Mission & vision:

Acquisition of Fair Hospital Limited

The proposed acquisition of Fair Hospital Limited aligns well with Astha General Hospital Limited's mission and vision to expand access to quality, affordable, and ethical healthcare nationwide. Acquiring an established hospital allows Astha to scale faster, enhance patient reach, and integrate new services without the delays of building from scratch. It also supports operational synergy through shared expertise, infrastructure, and cost efficiencies. However, potential challenges may arise from differences in management culture, clinical practices, or governance standards. To maintain brand integrity and uphold patient-centric values, Astha must ensure smooth cultural integration and consistent quality control post-acquisition.

Setting up the Astha Healthcare Network

Establishing the Astha Healthcare Network closely aligns with the company's mission to deliver accessible and integrated healthcare and its vision of building a collaborative medical ecosystem. Partnering with small and medium hospitals and diagnostic centres under the Astha brand expands reach efficiently, strengthens quality control, and promotes shared technology platforms such as unified patient records. This model fosters inclusivity supports community-level healthcare, and leverages Astha's reputation without heavy capital requirements. Its success, however, depends on maintaining strict partner selection criteria, strong governance, and uniform service standards across all affiliated facilities to preserve trust and brand consistency.

5.5. Conclusion:

- The acquisition of Fair Hospital Limited yields a positive NPV of BDT 134,104,971, confirming strong financial viability and potential for long-term value creation.
- The Astha Healthcare Network Limited option generates a positive NPV of BDT 89,655,005, indicating commercial potential but lower financial returns compared to the hospital acquisition.
- Acquiring Fair Hospital Limited ensures immediate expansion into Chattogram, enabling faster regional diversification and nationwide presence for Astha.
- The acquisition provides operational synergies, optimized cost structures, and faster revenue realization through existing patient base and infrastructure.
- The Healthcare Network model offers a scalable, asset-light approach that broadens Astha's reach and promotes digital integration across partner facilities.
- Acquisition of Fair Hospital Limited directly aligns with Astha's mission to provide ethical, quality, and affordable healthcare by leveraging an established institutional setup.
- Setting up 'Astha Healthcare Network' also aligns with the organization's vision to build a collaborative, technology-driven healthcare ecosystem for long-term inclusiveness.
- Between the two, acquiring Fair Hospital Limited offers higher NPV, lower implementation risk, and stronger strategic fit with Astha's immediate growth priorities.
- The Healthcare Network model remains a promising secondary strategy to support future expansion once Astha strengthens its operational base.
- Based on financial returns, strategic alignment, and mission compatibility, Astha should proceed with the acquisition of Fair Hospital Limited and defer the network model for later execution.

5.6. Recommendation:

- Proceed with the acquisition of Fair Hospital Limited, as it demonstrates a positive NPV of BDT 134,104,971 and offers the highest financial return and strategic fit.
- Defer the establishment of Astha Healthcare Network Limited for a later phase, as it provides lower NPV and requires stronger operational capacity for effective implementation.
- Prioritize integration of Fair Hospital Limited into Astha's system to ensure seamless alignment in governance, service quality, and brand standards.
- Leverage Fair Hospital's existing infrastructure and patient base to accelerate regional expansion and revenue growth in Chattogram and other urban markets.
- Allocate resources to upgrade Fair Hospital's facilities, technology, and branding, ensuring consistency with Astha's quality, ethical, and patient-centric standards.
- Develop a strategic roadmap to expand Astha's healthcare network model in future, capitalizing on lessons from the hospital acquisition and established operational efficiency.
- Implement strong post-acquisition monitoring and KPIs to track financial performance, service quality, and cultural integration outcomes.
- Focus on scalable and sustainable healthcare growth, balancing immediate expansion through acquisition with long-term digital and network-based strategies.

6. Evaluation of viability of bank borrowing as financing option (Requirement b-iii):

As Astha has very low gearing ratio, it has been considering bank borrowing as one of the financing options. Astha's over reliance on equity is limiting growth opportunities and shareholder returns. However, financing investment project through bank borrowing would maximise return by sustainably increasing gearing position. According to this loan-financing option, Astha will borrow entire investment amount of BDT 4,000,000,000 to acquire Fair Hospital Limited.
Astha's bank has agreed to provide bank loan exceeding BDT 500,000,000 through syndication with another bank for an annual interest rate of 13%. Furthermore, Astha will have to pay 0.5% arranger fee and 0.2% participation fee on syndicated loan. This fee will be deducted at the time of loan disbursement. In addition, Astha will have to pay BDT 600,000 annually to its bank as an agency fee. Astha will repay 1/5th of the principal payment at the end of each year together with interest accrued in that year.

To receive the required fund, Astha has to borrow from bank an amount equal to
= {4,000,000,000/(100%-0.5%-0.2%)}
= (4,000,000,000/99.3%)
= 4,028,197,382

From this loan amount bank will deduct arranger fee of (4,028,197,382 x 0.5%) = 20,140,987 and participation fee of (4,028,197,382 x 0.2%) = 8,056,395 and disburse rest amount of BDT 4,000,000,000 which will be used for financing the Fair Hospital Limited acquisition.

Considering bank borrowing, revised NPV is calculated as below:

Particulars	2025	2026	2027	2028	2029	2030
Project Cash flows	**(4,000,000,000)**	**198,750,000**	**265,565,000**	**327,494,300**	**375,221,266**	**6,066,945,040**
Loan Receipt	4,028,197,382	(805,639,476)	(805,639,476)	(805,639,476)	(805,639,476)	(805,639,476)
Arranger Fee	(20,140,987)	-	-	-	-	-
Participation Fee	(8,056,395)	-	-	-	-	-
Agency Fee	-	(600,000)	(600,000)	(600,000)	(600,000)	(600,000)
Interest Expenses	-	(523,665,660)	(418,932,528)	(314,199,396)	(209,466,264)	(104,733,132)
Revised Cash Flow	**(0)**	**(1,131,155,136)**	**(959,607,004)**	**(792,944,572)**	**(640,484,474)**	**5,155,972,432**
PV Factor	1	0.8850	0.7831	0.6931	0.6133	0.5428
Present Value	**(0)**	**(1,001,072,295)**	**(751,468,245)**	**(549,589,883)**	**(392,809,128)**	**2,798,661,836**

NPV	**103,722,285**

Analysis shows that even though financing the investment option entirely through bank loan, NPV remains positive at BDT 103,722,285. Therefore, financing the acquisition of 'Fair Hospital Limited' through bank loan is a viable option which Astha's board may consider.

6.1. Conclusion:

- Financing the acquisition of Fair Hospital Limited entirely through bank borrowing results in a positive NPV of BDT 103.7 million, confirming the financial viability of this funding option.
- The use of 100% debt financing improves project returns relative to full-equity funding, allowing Astha to preserve internal funds for future strategic investments.
- Astha's low current gearing ratio provides sufficient borrowing capacity, enabling sustainable leverage without jeopardizing financial stability.
- The annual interest rate of 13%, though relatively high, remains manageable within projected cash flows, supported by Fair Hospital's strong operating profitability.
- The syndicated loan structure diversifies financing risk between two banks while facilitating access to the required large-scale funding.
- Although arranger, participation, and agency fees slightly increase the financing cost, their impact is marginal relative to the project's overall positive NPV.
- The equal annual principal repayments over five years align with the project's expected cash generation, reducing refinancing risk and maintaining steady debt reduction.
- Projected cash inflows are sufficient to meet both interest and principal obligations, ensuring the loan remains serviceable throughout the financing term.
- Debt financing allows Astha to enhance equity returns by leveraging its capital structure efficiently while retaining full ownership of Fair Hospital Limited.
- Overall, bank borrowing presents a financially sound and strategically aligned option, improving shareholder value, optimizing capital structure, and supporting Astha's growth objectives sustainably.

6.2. Recommendation:

- Proceed with financing the acquisition of Fair Hospital Limited through bank borrowing, as the debt-financed structure yields a positive NPV of BDT 103.7 million and enhances shareholder value.
- Utilize full bank loan financing under the proposed syndication arrangement, leveraging Astha's low gearing position to optimize capital efficiency while preserving internal equity for future expansion.
- Negotiate favourable terms within the 13% annual interest framework to secure flexibility in repayment and minimize overall financing cost over the five-year period.
- Establish a comprehensive debt servicing plan ensuring timely payment of annual interest and structured repayment of one-fifth of the principal each year.
- Monitor Fair Hospital's operational cash flows quarterly to confirm adequate coverage of debt obligations and early identification of any emerging liquidity pressure.
- Allocate a sinking fund or liquidity reserve from project cash flows to support scheduled principal repayments and maintain financial stability.
- Carefully review and negotiate loan covenants to avoid restrictive conditions that may limit Astha's operational or investment flexibility.
- Engage in regular communication with syndicate banks to maintain transparency and uphold Astha's credit reputation, facilitating future borrowing opportunities.
- Plan a gradual deleveraging strategy after the loan term to restore moderate gearing and ensure long-term solvency and balance sheet strength.
- Integrate this financing strategy within Astha's broader capital structure plan to maximize returns, sustain growth, and align with the company's mission of financially prudent expansion.

7. Expected share premium in IPO and number of new shares to be issued (Requirement b-iv):

7.1. Evaluation of acceptability of expected share premium by BSEC:

As an alternative financing option, Astha's board is considering raising funds by issuing a maximum 25% share in IPO. Board is expecting to raise fund with 500% premium subject to BSEC approval. As listed companies have per share face value of BDT 10 taka, share premium will be BDT 50 taka. So total expected issue price is BDT 60 per share. While approving share premium, BSEC considers the NAV, EPS and sectoral PE ratio with equal importance. From the provided information, average market price and EPSs of other comparable companies of the sector were as follows:

Comparable Company Name	NAV on March (BDT)	3-month average market price	EPS on March 2024
Shahin Healthcare PLC	83.60	87.30	6.04
Modern Specialized Hospital PLC	17.10	16.70	1.70
Spring Medicare PLC	48.30	69.10	1.56
Oasis Diagnostics PLC	33.70	23.80	2.75
Healing General Hospital PLC	142.00	224.90	23.60
Average	**64.94**	**84.36**	**7.13**

Share Premium Based on NAV:

Based on Astha's adjusted financial statements in annexure-1 of this report, total asset is BDT 12,792,659,046 and number of outstanding shares is (Share capital BDT 2,293,454,420/ BDT 10 per share) = 229,345,442 (weighted average of shares outstanding is ignored for simplicity). Therefore, the Astha's NAV is 55.78 per share. Based on average NAV and market price of the comparable companies, Astha's expected share price would be

= average market price x (Astha's NAV/Average NAV)

= 84.36 x (55.78 / 64.94) = 72.46 ≈ 72 (approximate)

Therefore, Astha's share price based on NAV method will be BDT 72 per share. In that case, share premium would be 72 – 10 = BDT 62 per share.

Share Premium Based on EPS:

Based on annexure-2 of this report, Astha's Net profit for the year is BDT 932,709,965. So, Astha's EPS is BDT 932,709,965/ 229,345,442 = 4.07.

Considering the EPS of comparable companies and their market price, Astha's indicating share price will be derived at:

= average market price x (Astha's EPS/Average EPS)
= 84.36 x (4.07/7.13) = 48.16 ≈ 48 approximately.

Therefore, maximum share premium would be 48 – 10 = BDT 38 per share.

Share Premium Based on Sectoral PE:

From the provided information, sector's price earnings (P/E) ratio is 12.2. As Astha's earnings per share is BDT 4.07, its estimated share price based on sectoral PE would be
= 12.2 x 4.07 = 49.65 ≈ 50 approximately.

Therefore, maximum share premium would be 50 - 10 = BDT 40 per share.

Acceptable Share Price by BSEC:

From the Analysis it appears that considering all NAV, EPS and Sectoral PE with equal weight, BSEC is likely to approve share price of (75+48+50)/3 = 57.67 ≈ BDT 58 approximately per share. That means maximum share premium that BSEC will accept is 58 – 10 = 48. As Astha management is expecting to raise share premium of BDT 50, it is likely that BSEC will not approve the IPO with 500% share premium. However, it would likely to approve up to 480% share premium.

7.2. Number of new shares to be issued:

Astha will require to raise BDT 4,000,000,000 after 3% issue cost in IPO at share price of BDT 58 including 480% share premium. Therefore, total number of new shares to be issued is:

= [4,000,000,000 / (1-0.3)]/58 = 5,714,285,714 / 58 = 98,522,167 new shares.

Astha at present has 229,345,442 outstanding shares. After IPO, total number of shares will be = (229,345,442+ 98,522,167) = 327,867,609 shares. Post IPO, Astha's existing shareholders will own (229,345,442/ 327,867,609) = 69.95% shares. That means Astha will issue 30.05% shares in IPO which is above the acceptable limit of 25% set by board of directors.

7.3. Conclusion:

- Astha is considering raising BDT 4,000 million through an IPO by issuing up to 25% of its shares, subject to BSEC approval.
- The board proposed a 500% share premium (BDT 50 over the face value of BDT 10), resulting in an expected issue price of BDT 60 per share.
- BSEC determines fair value based on equal weighting of NAV, EPS, and sectoral P/E ratio; hence, the proposed premium must align with market benchmarks.
- Astha's NAV per share is BDT 55.78, which yields a fair value of BDT 72 and a corresponding share premium of BDT 62 based on peer NAV comparison.
- Based on Astha's EPS of BDT 4.07, fair valuation through comparable EPS suggests a share price of BDT 48, implying a share premium of BDT 38.
- Applying the sectoral P/E ratio of 12.2, the share price is estimated at BDT 50, translating into a share premium of BDT 40.
- By equally weighting the NAV, EPS, and P/E-based methods, the fair value is approximately BDT 58 per share, suggesting a BSEC-acceptable premium of BDT 48.
- The proposed 500% premium is unlikely to be accepted; however, a 480% premium (BDT 48) is considered reasonable and likely to gain regulatory approval.
- At a share price of BDT 58, Astha will need to issue approximately 98.52 million new shares to raise BDT 4 billion (net of issue cost), increasing total shares to 327.87 million.
- Following the IPO, existing shareholders would retain 69.95% ownership, while public investors would hold 30.05%, exceeding the board-approved ceiling of 25%.
- The IPO remains a viable financing option if the premium is revised to align with BSEC valuation and the issue size is reduced to maintain the 25% ownership limit.

7.4. Recommendation:

- Revise the proposed share premium around BDT 48 per share (480%) instead of the initially proposed 500%, ensuring regulatory approval and market acceptance.
- Adjust the issue size to maintain compliance with the board-approved 25% ownership ceiling by recalibrating the number of shares to be issued while achieving the required BDT 4 billion target.
- Engage a BSEC-accredited issue manager and underwriter early in the process to strengthen documentation, manage regulatory queries, and ensure transparent valuation presentation.
- Conduct an independent fair value assessment by a credible valuation firm to support the proposed share price and justify the premium through objective, data-driven analysis.
- Enhance financial reporting transparency and strengthen internal controls prior to the IPO to increase investor confidence and reduce audit-related risks during regulatory review.
- Develop a detailed utilization plan for the IPO proceeds, clearly identifying project-wise fund allocation, expected ROI, and implementation timelines to demonstrate financial prudence.
- Improve corporate governance practices by appointing at least one independent director and formalizing an audit committee to meet BSEC's public company governance standards.
- Engage in pre-IPO investor roadshows and institutional outreach programs to build brand awareness, gauge investor sentiment, and ensure successful market response.

8. Comment on the ethical issues involving Astha's business (Requirement C-i):

8.1. Ethical issues:

Several ethical issues have been identified in relation to Astha General Hospital Limited's current operations and future plans involving its employees, suppliers, customers, and broader stakeholder relationships. These concerns centre around transparency, fairness, governance integrity, and corporate responsibility:

- **Employment of Underqualified or Overworked Staff**
  Astha's rapid expansion has led to high employee turnover and overworked clinical and administrative staff, particularly in emergency and diagnostic units. This raises ethical concerns about workplace well-being, staff burnout, and patient safety in a healthcare environment that demands continuous vigilance.
- **Potential Conflict of Interest in External Audit Appointment**
  The hospital continues to engage an external auditor connected to a senior finance executive despite the firm not being enlisted with the Financial Reporting Council (FRC). This represents a breach of audit independence and governance ethics for a Public Interest Entity, risking credibility of reported financial information.
- **Patient Privacy and Data Protection Gaps**
  Astha's Hospital Management System (HMS) and "Astha Card" store sensitive patient and billing information, yet there is no formal data protection or consent policy in place. Lack of clear digital governance over personal health data exposes the organization to ethical and legal risks regarding confidentiality and misuse of patient information.
- **Equity and Transparency in Patient Services**
  Reports of inconsistent service quality between Dhaka and branch hospitals indicate a disparity in patient care standards. Unequal access to experienced doctors, diagnostic accuracy, and support services reflect an ethical gap in delivering equitable healthcare across all facilities.
- **Supplier Relationship and Procurement Ethics**
  Astha's procurement process relies heavily on long-term supplier credit and consignment arrangements. However, limited due diligence on supplier labor conditions, waste management, and pricing fairness raises ethical questions regarding responsible sourcing and sustainability compliance.
- **Governance Concentration and Lack of Independent Oversight**
  Astha's board is dominated by family members and senior executives with overlapping decision-making authority. The absence of independent directors or external oversight creates ethical vulnerabilities related to nepotism, self-interest, and weak accountability structures in strategic and financial decisions.
- **Patient Communication and Transparency**
  Instances of billing disputes, opaque pricing structures for medical tests, and delayed refund policies suggest a lack of transparency in patient communication. Ethical healthcare delivery requires clear disclosure of costs and procedures, which is currently inconsistent across departments.
- **Expansion without Proportionate Quality Safeguards**
  The hospital's ongoing expansion into new cities and services, despite liquidity constraints and limited managerial bandwidth, risks compromising service quality, patient safety, and long-term institutional integrity. Ethically, rapid expansion should not come at the expense of care quality and staff welfare.

8.2. Conclusion

- The overwork and high turnover of clinical and support staff raise ethical concerns around employee welfare, patient safety, and the hospital's responsibility to maintain a fair and humane work environment.
- The continued engagement of an audit firm linked to a senior executive and not registered with the FRC undermines independence, transparency, and compliance with Public Interest Entity governance standards.
- The absence of clear patient data protection and consent protocols within Astha's Hospital Management System and "Astha Card" program presents serious risks to privacy, confidentiality, and ethical handling of sensitive health information.
- Disparities in patient service quality and resource allocation between Dhaka and regional branches highlight inequities in healthcare delivery and compromise Astha's ethical commitment to equal patient care.
- Heavy reliance on supplier credit without due diligence on ethical sourcing, labor conditions, or sustainability exposes Astha to reputational and compliance risks across its supply chain.
- The family-dominated management and lack of independent directors weaken governance transparency, increasing the potential for conflicts of interest and undermining institutional accountability.
- Opaque billing practices, inconsistent communication on medical costs, and refund delays suggest lapses in ethical patient engagement and transparency in financial dealings.
- Aggressive expansion amid liquidity pressures and thin managerial oversight raises ethical questions about prioritizing growth over patient care quality, financial prudence, and long-term institutional sustainability.

8.3. Recommendation

- Strengthen employee welfare policies by addressing staff overwork and turnover through fair workload distribution, adequate rest periods, and improved compensation aligned with healthcare labor standards.
- Appoint an independent, FRC-enlisted external auditor and establish an internal audit committee to ensure audit independence, compliance, and transparent financial reporting as required for a Public Interest Entity.
- Develop and implement a comprehensive patient data protection and privacy policy, ensuring informed consent, secure data storage, and responsible use of patient and customer information.
- Standardize clinical and administrative staff training across all hospital units to ensure consistent service quality, ethical patient care, and equitable investment in employee development.
- Introduce independent directors and governance committees to reduce family dominance, enhance accountability, and align corporate decision-making with ethical and regulatory expectations.
- Establish a supplier code of ethics emphasizing fair trade, safe labor practices, and environmental sustainability; revise credit and consignment policies to ensure equitable treatment of SME suppliers.

- Ensure transparent patient communication by introducing standardized billing disclosures, cost estimates before treatment, and simplified refund and complaint mechanisms.
- Expand and formalize health and safety audit programs, with regular inspections, employee training, and public disclosure of compliance results to safeguard workplace and patient safety.
- Align expansion plans with financial capacity by moderating new project rollouts, conducting risk-based feasibility studies, and prioritizing the performance optimization of existing hospitals.
- Introduce a corporate ethics and compliance framework, including a whistleblower policy, annual ethics training, and monitoring mechanisms to promote accountability and an ethical organizational culture.

9. Assessment of sustainability factors of Astha's business (Requirement C-ii).

A business can earn sustainability when it achieves Human Sustainability, Social Sustainability, Economic Sustainability and Environmental Sustainability. Astha's business has potential to contribute significantly to human, social, economic, and environmental sustainability. Below is an analysis based on these dimensions:

9.1. Human Sustainability:

- **High Employee Turnover and Workforce Fatigue:**
  Astha continues to experience high staff turnover, particularly among nurses, technicians, and junior administrative personnel. Heavy workloads, long shifts, and limited career progression opportunities contribute to employee dissatisfaction and fatigue, threatening service continuity and overall patient care quality.
- **Overdependence on Contractual and Temporary Staff:**
  To manage costs and operational flexibility, Astha relies significantly on temporary and outsourced personnel in non-clinical areas. This approach reduces long-term labour costs but compromises employee loyalty, institutional knowledge retention, and consistent service standards across departments.
- **Workplace Health and Safety Gaps:**
  Although Astha maintains basic occupational safety measures, including emergency response and infection control protocols, compliance monitoring remains limited. The lack of systematic risk assessments and preventive maintenance exposes staff to potential workplace hazards, especially in laboratory and waste management areas.
- **Inconsistent Training and Skill Development:**
  Astha's staff training programs vary widely in content and delivery between main and branch hospitals. The absence of a unified learning framework for clinical and administrative personnel results in uneven service quality and lower efficiency in newly established units.
- **Insufficient Employee Well-being and Mental Health Support:**
  Healthcare workers face high-stress environments, yet Astha lacks formal programs addressing staff well-being, counselling, or mental health support. This neglect undermines human sustainability by reducing job satisfaction, increasing absenteeism, and raising burnout risks.
- **Limited Diversity, Inclusion, and Equal Opportunity Policies:**

There is little evidence of structured diversity initiatives or gender equality frameworks within Astha's HR policies. Leadership and mid-management positions are male-dominated, and the absence of inclusion programs limits opportunities for female and differently abled professionals.

- **Ethical Employment and Labor Practices:**
  Although Astha follows standard labour laws, inconsistencies in shift scheduling, unpaid overtime, and irregular contract renewals have been reported. Establishing transparent labour policies and ensuring compliance with labour rights would strengthen ethical employment practices.

9.2. Social Sustainability

- **Positive Social Impact through Accessible Healthcare:**
  Astha General Hospital Limited contributes significantly to community health by providing affordable medical, diagnostic, and pharmacy services in urban areas. Its 24/7 emergency care, vaccination drives, and free medical camps demonstrate a commitment to social welfare and public health improvement, particularly for middle-income and lower-middle-income groups.
- **Commitment to Community Development through CSR:**
  The hospital actively engages in CSR initiatives focused on education, healthcare, and skill-building for underprivileged groups. Programs such as health awareness campaigns and blood donation drives reflect genuine social contribution; however, the lack of formal impact measurement limits understanding of their effectiveness and scalability.
- **Limited Outreach to Underserved Regions:**
  While Astha's services are highly valued in Dhaka and major cities, its limited presence in rural and semi-urban areas restricts access to quality healthcare for marginalized populations. Expanding through satellite clinics or partnerships with local health centres could enhance inclusivity and equitable service distribution.
- **Employee and Labor Relations Affecting Social Value:**
  High staff turnover and inconsistent training impact not only internal efficiency but also the quality of care delivered to patients which is a key element of social sustainability in healthcare. Strengthening workforce engagement, continuous professional development, and fair labour practices are essential to maintain patient trust and community confidence.
- **Public Trust and Ethical Reputation:**
  Astha's reputation as a trusted private healthcare provider strengthens its social capital. However, controversies over workforce ethics and audit independence have attracted public scrutiny, underscoring the need for transparent governance to maintain credibility and social legitimacy.
- **Partnership and Local Economic Contribution:**
  By sourcing medical supplies and services locally and collaborating with SMEs, Astha supports domestic economic activity. However, formalizing supplier standards on labour, safety, and ethical sourcing would reinforce its social accountability and sustainable procurement practices.
- **Patient Engagement and Feedback Mechanisms:**
  Although Astha maintains a strong patient base, formal systems for capturing feedback and grievances are underdeveloped. Establishing structured community engagement and satisfaction assessment frameworks would improve patient experience and reinforce Astha's social responsibility in healthcare delivery.

9.3. Economic Sustainability

- **Steady Revenue Growth but Modest Profitability:**
  Astha's adjusted revenue grew by 13.2% in FY2025 to BDT 3.98 billion, supported by strong hospital and diagnostic performance. However, a 3.10% net profit margin, though improved from 0.10%, reflects limited conversion of revenue into sustainable earnings.
- **Stronger Capital Structure and Low Debt Exposure:**
  With a debt-to-equity ratio reduced to 0.02, Astha has adopted a conservative financing approach that enhances stability but limits leverage-driven expansion potential.
- **Improved Cost Control and Efficiency:**
  Gross profit rose 14.7%, outpacing revenue growth, indicating effective management of medical and logistics costs despite inflationary pressures.
- **Underutilized Assets and Low Return Ratios:**
  ROA (0.97%) and ROE (1.12%) remain weak due to unproductive or newly added facilities yet to achieve full operational capacity.
- **Tightening Liquidity Position:**
  A current ratio of 1.39 and quick ratio of 1.14 indicate adequate short-term solvency but highlight the need for better cash flow and receivable management.
- **Financial Resilience Amid Economic Challenges:**
  Astha maintained positive cash flow and profitability despite inflation, political instability, and rising expenses which are demonstrating solid financial governance and adaptability.
- **Balanced Investment Strategy Needed:**
  Future sustainability depends on optimizing asset use, reinvesting in digital infrastructure, and maintaining profitability before further expansion.

9.4. Environmental Sustainability

- **Growing Awareness but Limited Green Initiatives:**
  Astha has acknowledged environmental responsibility through basic waste segregation and hygiene practices but lacks a structured environmental management policy or sustainability roadmap across its facilities.
- **Biomedical and Waste Management Compliance:**
  The hospital disposes of medical and hazardous waste through certified vendors, ensuring partial compliance with health and environmental standards. However, inconsistent monitoring and limited vendor oversight may expose gaps in safe disposal and reporting.
- **High Energy Consumption and Carbon Footprint:**
  Diagnostic equipment, air conditioning, and refrigeration systems contribute significantly to electricity use. The absence of renewable energy initiatives such as solar integration or energy-efficient lighting increases operational costs and environmental impact.
- **Water Usage and Wastewater Concerns:**
  Astha relies heavily on municipal water systems without a formal wastewater treatment facility. This raises sustainability concerns related to water efficiency, contamination control, and regulatory compliance.

- **Limited Sustainability Investments Due to Cost Constraints:**
  Budget limitations have delayed the adoption of eco-friendly technologies such as energy-efficient machinery, green building materials, or smart energy management systems.
- **Opportunity for Sustainable Transformation:**
  By introducing environmental audits, expanding waste recycling, and adopting renewable energy solutions, Astha can significantly reduce its ecological footprint while aligning with global ESG healthcare standards.

9.5. Conclusion

- High employee turnover, workload imbalance, and inconsistent training across hospital branches highlight weaknesses in human sustainability, reducing staff morale and long-term workforce stability.
- Absence of structured well-being and mental health programs for healthcare professionals undermines employee satisfaction and resilience in a high-stress hospital environment.
- Limited diversity and inclusion measures within Astha's workforce restrict fair representation, particularly for women and differently abled employees, hindering the development of a progressive workplace culture.
- Astha's CSR and community engagement programs, including free medical camps and vaccination drives, reflect positive social intent but lack measurable outcomes and transparent reporting, limiting their long-term social value.
- Urban-centric operations and limited rural outreach restrict equitable healthcare access, reducing Astha's contribution to inclusive national health development and social sustainability.
- Weak supplier monitoring and lack of ethical sourcing policies create potential social and reputational risks in procurement, as SME suppliers bear most of the financial and operational burden.
- Economic sustainability remains moderate, with strong revenue growth and improved cost control offset by low profitability, underutilized assets, and tightening liquidity.
- Low leverage and prudent financial management reflect stability but also limited strategic investment in modernization and capacity expansion that could enhance long-term economic performance.
- Environmental sustainability is underdeveloped, with high energy consumption, inadequate waste treatment, and limited investment in renewable or eco-efficient technologies.
- Astha's overall sustainability performance is mixed with financially resilient and socially active but constrained by weak human development, limited community inclusiveness, and minimal environmental commitment. Building a unified ESG framework will be critical for achieving balanced and responsible long-term growth.

9.6. Recommendation

- Strengthen employee development and retention programs by introducing structured training, fair workload policies to reduce turnover and improve workforce stability.
- Implement formal health, safety, and well-being initiatives including regular safety audits, stress management, mental health support programs to protect staff welfare and maintain service quality.

- Adopt a gender equity and inclusion framework promoting equal opportunities for women and differently abled employees, ensuring diversity in leadership and medical roles.
- Establish measurable CSR and community health programs with clear performance indicators, periodic public reporting, and third-party evaluations to demonstrate tangible social impact.
- Expand access to healthcare in rural and semi-urban regions through outreach centres, telemedicine services, and partnerships with local healthcare providers to promote national health coverage.
- Introduce a supplier code of conduct covering ethical sourcing, fair labour practices, and sustainability compliance to ensure accountability across the procurement chain.
- Optimize financial performance through asset utilization and cost efficiency, balancing expansion with profitability to sustain long-term economic growth and investor confidence.
- Leverage low debt levels strategically to invest in modernization, automation, and technology-driven healthcare solutions that enhance operational efficiency and patient outcomes.
- Develop and implement an environmental management policy addressing waste reduction, renewable energy adoption, and water efficiency to minimize environmental footprint.
- Integrate sustainability governance into corporate strategy by forming an ESG committee, aligning policies with international standards, and embedding sustainability reporting within annual disclosures.

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10. Appendices:

10.1. Appendix 1 - Analysis of Adjusted Balance Sheet:

	30-Jun-25	Adjustments	Adjusted 2025	Restated 2024
ASSETS				
Non-current assets				
Property. plant and equipment	11,569,276,404	(75,000)	11,569,201,404	10,308,172,594
Right of use assets	8,399,231	0	8,399,231	25,197,609
Investment in associates	62,959,202	0	62,959,202	62,526,051
	11,640,634,837	**(75,000)**	**11,640,559,837**	**10,395,896,254**
Current assets				
Inventories	210,701,892	0	210,701,892	217,224,608
Trade and other receivables	98,167,138	0	98,167,138	105,707,506
Advance. deposits and prepayments	288,363,099	0	288,363,099	280,516,610
Cash and cash equivalents	554,867,080	0	554,867,080	466,732,824
	1,152,099,209	**0**	**1,152,099,209**	**1,070,181,548**
Total assets	**12,792,734,046**	**(75,000)**	**12,792,659,046**	**11,466,077,802**
EQUITY AND LIABILITES				
Shareholders' Equity				
Paid-up capital	2,293,454,420	0	2,293,454,420	2,084,958,540
Retained earnings	2,026,119,960	(817,823,394)	1,208,296,566	1,293,299,208
Re-measurement reserve	(178,256)	(1,727,465)	(1,905,721)	(178,256)
Revaluation reserve	6,699,409,047	810,944,192	7,510,353,239	6,699,409,047
	11,018,805,171	**-8,606,667**	**11,010,198,504**	**10,077,488,539**
Non-current liabilities				
Deferred tax liabilities	888,548,703	40,117,238	928,665,941	447,743,934
Gratuity obligations	17,199,796	6,546,784	23,746,580	17,199,796
Lease liability	-	-	0	7,802,063
Long term loan	2,740,723		2,740,723	192,811,382
	908,489,222	**46,664,022**	**955,153,244**	**665,557,175**
Current liabilities				
Long term loan- current portion	225,969,600	0	225,969,600	225,601,400
Lease liability- current portion	9,367,892	0	9,367,892	19,087,488
Trade and other payables	371,411,030	0	371,411,030	344,771,928
Provision and accruals	258,691,131	(38,132,355)	220,558,776	133,571,272
	865,439,653	**(38,132,355)**	**827,307,298**	**723,032,088**
Total liabilities	**1,773,928,875**	**8,531,667**	**1,782,460,542**	**1,388,589,263**
Total equity and liabilities	**12,792,734,046**	**(75,000)**	**12,792,659,046**	**11,466,077,802**

10.2. Appendix 2 - Analysis of Adjusted Statement of Profit & Loss:

	2025	Adj	Adjusted 2025	Restated 2024	Change	% Change
Revenue	3,980,671,132	-	3,980,671,132	3,516,474,091	464,197,041	13.2%
Direct expenses	(1,614,813,299)	-	(1,614,813,299)	(1,453,254,366)	(161,558,933)	11.1%
Gross profit	**2,365,857,833**	**-**	**2,365,857,833**	**2,063,219,725**	**302,638,108**	**14.7%**
Administrative expenses	(2,188,535,154)	(5,713,992)	(2,194,249,146)	(2,037,595,715)	(156,653,431)	7.7%
Profit from operation	**177,322,679**	**(5,713,992)**	**171,608,687**	**25,624,010**	**145,984,677**	**569.7%**
Non-operating income	920,356,897	(853,625,465)	66,731,432	70,028,979		
Financial expenses	(57,200,354)	(1,289,985)	(58,490,339)	(64,469,100)		
Profit before WPPF Exp.	**1,040,479,222**	**(860,629,442)**	**179,849,780**	**31,183,889**		
Contribution to WPPF	(49,546,630)	40,982,355	(8,564,275)	(1,630,453)		
Profit before tax	**990,932,592**	**(819,647,087)**	**171,285,505**	**29,553,436**	**141,732,069**	**479.6%**
Current income tax	(60,704,008)	-	(60,704,008)	(27,887,384)		
Deferred income tax	11,088,048	1,823,693	12,911,741	1,940,391		
Profit after tax	**941,316,632**		**123,493,238**	**3,606,443**	**119,886,795**	**3324.2%**
Other comprehensive income	-	809,216,727	809,216,727	(178,256)		
Total comprehensive income	**941,316,632**	**809,216,727**	**932,709,965**	**3,428,187**		

10.3. Appendix 3 - Mix analysis:

Based on the reported revenue and adjustments:

	2025	2024	Growth	Growth %
Revenue Mix Analysis				
Medical Services	1,837,172,814	1,549,235,374	287,937,440	18.6%
Diagnostic Services	1,028,480,930	926,764,776	101,716,154	11.0%
Pharmacy	1,115,017,388	1,040,473,941	74,543,447	7.2%
	3,980,671,132	**3,516,474,091**		
Revenue Contribution				
Medical Services	46.2%	44.1%		
Diagnostic Services	25.8%	26.4%		
Pharmacy	28.0%	29.6%		
Cost of Revenue Analysis				
Medical Services	354,361,529	309,034,973	45,326,556	14.7%
Diagnostic Services	411,384,926	367,248,971	44,135,955	12.0%
Pharmacy	849,066,844	776,970,422	72,096,422	9.3%
	1,614,813,299	**1,453,254,366**		
Cost of Revenue Contribution				
Medical Services	21.9%	21.3%		
Diagnostic Services	25.5%	25.3%		
Pharmacy	52.6%	53.5%		

	2025	2024	Growth	Growth %
Gross Profit Analysis				
Medical Services	1,482,811,285	1,240,200,401	242,610,884	19.6%
Diagnostic Services	617,096,004	559,515,805	57,580,199	10.3%
Pharmacy	265,950,544	263,503,519	2,447,025	0.9%
	2,365,857,833	**2,063,219,725**		
Gross Profit Contribution				
Medical Services	62.7%	60.1%		
Diagnostic Services	26.1%	27.1%		
Pharmacy	11.2%	12.8%		

10.4. Appendix 4: Ratio Analysis:

Sl.	Ratio Name	Formula Used	30-Jun-25	30-Jun-24
1	Current Ratio	$\frac{\text{Current Assets}}{\text{Current Liabilities}}$	1.39	1.48
2	Quick Ratio	$\frac{\text{(Current Assets-Inventories)}}{\text{Current Liabilities}}$	1.14	1.18
3	Debt to equity ratio	$\frac{\text{Interest bearing loans}}{\text{Total Equity}}$	0.02	0.04
4	Asset Turnover Ratio	$\frac{\text{Revenue}}{\text{Total Assets}}$	0.31	0.31
5	TIE Ratio	$\frac{\text{(PBT + financing Expenses)}}{\text{Financing Expenses}}$	3.93	1.46
6	Return on Asset	$\frac{\text{Profit for the year}}{\text{Total Assets}}$	0.97%	0.03%
7	Return on Equity	$\frac{\text{Profit for the year}}{\text{Total Equity}}$	1.12%	0.04%

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